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Health Sector Development Program IV

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Introduction

The major health problems of the country remain largely preventable communicable diseases and nutritional disorders. Despite major progresses have been made to improve the health status of the population in the last one and a half decades, Ethiopia's population still faces a high rate of morbidity and mortality and the health status remains relatively poor. For instance, more than 90% of child deaths are due to pneumonia, diarrhea, malaria, neonatal problems, malnutrition and HIV/AIDS, and often a combination of these conditions.

Following the changes of Government in 1991, the Government produced the health policy which was the first of its kind in the country and was among a number of political and socio-economic transformation measures that were put in place. The translation of the health policy was followed by the formulation of four consecutive phases of comprehensive Health Sector Development Plans (HSDPs), the first phase of which was implemented starting in 1996/97.

The basic steps that has used to prepare HSDP IV using BSC includes: a) Conduct Organizational assessment, b) Setting of Strategic elements, c) Setting of Strategic Objectives, d) Prepared Strategic Map of the Objectives, e) Indicated performance measures & Targets of the strategic plan, and f) Prepared the Strategic initiatives that have expected to achieve the overall strategic plan

In this document, we will discuss about the historical development and an overview of the development of the previous HSDP, and about their achievements and implementation challenges. We will also further discuss in detail; the guiding principles, national and international policy framework, strategic assessments, the HSDP IV strategic objectives (including strategic themes, results and the priorities), performance measures and initiatives of HSDP IV, costing and financing, and management arrangement of HSDP IV references.

Chapter 1: Overview of HSDP I, II, and III

There were encouraging improvements from the implementation of HSDP I and II, as shown both in the health service coverage as well as in the utilization of services at all levels of the health care system of Ethiopia.

Areas of improvement	Before (1996/97)	End of HSDP II (2003/04)
Health Posts, Health Centres, Hospitals	76; 243; 87	3,135; 519; 126
Contraceptive coverage	4%	25%

There have also been significant increases in the availability of health workers of all professional categories. Priority health programs, including prevention and control of infectious communicable diseases such as HIV/AIDS, Malaria, and TB, have achieved notable improvements during these periods. We will also see a few areas in which the predecessor HSPD III performed.

1.1 Maternal Newborn, Child, and Adolescent Health

Areas of improvement	Before HSDP III		End of HSDP III
	Coverage	Goal	
CPR	25%	60%	56.2%
SBA	12%	32%	18.4%
BEmONC	-	100%	14%
CEmONC	-	87% hospitals	51% hospitals
Penta vaccination	70%	85%	82%
Full immunization	45%	80%	65.5%
IMNCI	36%	90%	68.2% HC and 72.9% H

1.2 Malaria and Other Vector born Diseases Prevention and Control

Areas of improvement	Before HSDP III		End of HSDP III
	Coverage	Goal	
ITN distribution	-	20 M	22.2 M
Under 5 ITN usage	2.8%	63%	41.2%
Pregnancy ITN usage	1.6%	49%	42.5%
Under 5 mortality	5%	2%	4.5%
Above 5 mortality	4.5%	2%	3.3%

1.3 Nutrition

Nutritional disorders are the main causes of morbidity and mortality. The major problems are protein-energy malnutrition and micronutrient deficiencies such as vitamin A, iron, and Iodine. To achieve HSDP target of creating access for 90% of children 6-59 months for nutritional screening, nutritional screening is underway every three months at HP level with plan to screen more than 95% of the 6-59 months children.

- Infants 0-5 months exclusively breast fed from 38% to 63%, a study done in four regions showed that the prevalence of exclusive breast feeding has reached 76% in 2008/09.

- The proportion of children aged 6-59 months getting vitamin A prophylaxis through two rounds of Vitamin A supplementation per year reached 95% in 2008/09, far above HSDP III target of increasing from 38% to 54%.

1.4. Health Facility Construction and Expansion

Facility	HSDP I (1996/7)	HSDP II (2003/2004)	HSDP III (2010)
HP	76	2,899	14,416
HC	412	519	2689
Hospital	87	126	195

1.5. Pharmaceutical services

In order to introduce efficiency in the supply chain of pharmaceuticals and medical supplies management system, PHARMID has been transformed into Pharmaceutical Fund and Supply Agency (PFSA) with the several measures taken to strengthen the capacity of the new agency. These measures include:

- Deployment of more regular staff and mobilization of TAs.
- Design of the LMIS
- The selection of 18 sites and beginning of the construction of warehouses and hubs.
- Overhauling and strengthening the transport capacity of the Agency through the procurement of 92 trucks.
- Improvement in the RDF volume by making available additional funding.
- Building cold rooms that has increased the national capacity by fivefold.

Chapter 2: Health sector strategic analysis

2.1. Mission:

To reduce morbidity, mortality, and disability and improve the health status of the Ethiopian people through providing and regulating a comprehensive package of promotive, preventive, curative, and rehabilitative health services via a decentralized and democratized health system.

2.2. Core values:

1. Community first
 - We are here for nothing but to serve and satisfy our community.
 - We treat them as we would like to be treated.
 - We have three priorities: Community, Community, and Community.
2. Collaboration
 - We work together in a spirit of mutual support and understanding to achieve our collective goals.
3. Commitments
 - No matter what challenges we face and discomforts we feel, we stand firm, be patient, and exert our utmost and sustained effort to achieve our goals.
4. Change
 - We innovate new ways of doing things and will be open-minded to reforms.
5. Trust

- We ensure maximum vulnerability and integrity to each other.
6. Continued Professional Development
- Education, professional behavior and ethics, competence, and performance in work duties

2.3. SWOT Analysis of the Health Sector

Strength	Weakness
• High coverage of Health Extension Program • Adoption of cost effective strategies (RBM, MPS, IMCI, DOTS, etc • Successes in the prevention and control of malaria • Increased coverage of ART • Sustained High coverage of EPI • Accelerated training of Health professionals • Rapid expansion of health centers and health posts • Finalization of BPR in the sector • Commencement in the use of IT for telemedicine, tele-education, electronic medical records etc • Increased supply of medical equipment • Increased allocation and expenditure on health • Improvement in the harmonization and alignment • One plan streamlined in the sector • MDG Fund established to enforce one budget progress in the national HMIS scaling up as part of one M&E framework • Well defined HSDP Governance (CJSC, RJSC,etc)	• Inadequate capacity to implement decentralized health system • Inadequate follow-up on implementation of policies, guidelines, standards and protocols • Low utilization of health services • Weak referral system; • Slow and erratic implementation of BPR • Outdated health facility standards & Low health service quality • Low coverage of Skilled delivery & Newborn care, TB case detection, PMTCT • Inadequate attention to NCDs and NTDs • Inadequate management and support to PMTCT services, TB Leprosy control at sub-national levels. • Inadequate water supply and WASH to Health facilities, schools, and communities • Inadequate capacity of preparedness to efficiently respond to threats of epidemics such as Malaria. • Lack of Health infrastructure maintenance capacity (Building, Medical equipment, IT) • Shortage of drugs, medical supplies, equipment, and commodities & Slow implementation of Health Commodities Supply System (HCSS) • Weak regulatory system of pharmaceuticals and medical supplies • Inadequate coordination mechanism for public private partnership in health • Weak implementation of HSDP governance (such as the CJSC) • Inadequate quality of trainings and Low performance in training and deployment of model families; • Lack of HR motivation and retention strategy and absence of standardized CPD programs • Shortage and attrition of highly skilled professionals and attrition of HEWs ; • Lack of career development for HEWs & Poor coordination in the in-service trainings to HEWs by various partners. • Weak M&E and use of information for evidence-based decision making at lower levels • Persistently inadequate and inequitable distribution of resources compared to needs and priorities (Eg Health system, MNCH, & Health Infrastructure Expansion)
Opportunity	Threat

• Availability of coherent and pro-poor development policy including health • Democratization and decentralization of the health system • Availability of sound health policy • Increasing community participation • Increasing national income; • Health is accorded priority by the Government • Increasing human resource output via the capacity Building Programme • Increasing domestic manufacturing capacity of drugs; • Increasing education of girls • Emerging global health initiatives • Expansion of the private sector • Increasing external resources and TA • Expansion of infrastructure (road, ITC, electricity)	• Poverty and high level of population growth • Instability of neighboring countries- • Newly emerging pandemics • Manmade and natural disasters • Adult –illiteracy • Global Financial crisis (rising cost of pharmaceuticals, supplies etc) • Poor aid effectiveness • Brain drain, high turnover of health staff, especially medical doctors
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Chapter 3: Strategy of the HSDP- IV

As part of Ethiopia’s vision to become a middle-income country in the years following the MDG 2015 period, the health sector must adopt ambitious goals to ensure access to effective health services for all. A strong health strategy should clearly link system challenges with strategic interventions, required resources, and expected results while also defining the value provided to citizens, key strategic priorities, and performance perspectives.

3.1. Customer value proposition

A central element of the strategy is the **customer value proposition**, which defines what health services should be provided, the principles guiding the sector’s engagement with communities, and how the sector intends to be perceived. It focuses on delivering accessible, timely, high-quality healthcare within a safe and empowering environment. The health sector aims to present itself as transparent, professional, trustworthy, and customer-friendly, while fostering a relationship with the community that is cooperative, respectful, equitable, and rooted in mutual understanding and stewardship.

3.2. Strategic Themes, Strategic Results and Priorities

The strategy is built on three core **strategic themes** or "Pillars of Excellence": health service delivery, leadership and governance, and health infrastructure and resources.

The first theme, **Excellence in Health Service Delivery**, targets the delivery of preventive, curative, rehabilitative, and emergency services, as well as the promotion of personal and community health. It emphasizes maternal, neonatal, child, youth, and adolescent health; communicable and non-communicable disease prevention; nutrition; hygiene; and public health emergency response. The intended result is a community that is health-conscious, protected from emergencies, and has consistent access to quality care. Success will be measured through improved health practices, reduced morbidity and mortality, and better emergency preparedness.

The second theme, **Excellence in Leadership and Governance**, focuses on evidence-based planning, policy-making, and monitoring, along with the equitable and efficient allocation of resources. It seeks to create institutions that are transparent, accountable, and responsive to community needs. This includes empowering employees, enhancing public safety, and fostering partnerships. Achievements will be reflected in better use of health data, improved decision-making, and stronger stakeholder involvement.

The third theme, **Excellence in Health Infrastructure and Resources**, centers on developing and maintaining health facilities, medical equipment, and ICT networks, and ensuring the presence of qualified and motivated health professionals. It also includes strengthening pharmaceutical supply chains and health financing mechanisms. The goal is to ensure all communities have access to well-equipped, well-staffed, and digitally connected facilities. Performance will be gauged through indicators such as drug availability, equipment functionality, skilled workforce retention, and expanded insurance coverage.

3.3. Strategic Perspectives

The strategy is supported by four key **strategic perspectives** that guide how success will be achieved: community, financial, internal processes, and capacity building. These perspectives frame critical questions about empowering communities, optimizing resource use, improving service quality and responsiveness, and enhancing sector-wide capacity for long-term growth.

Chapter 4: Strategic objectives for the Ethiopian health sector

The strategic objectives outlined in this section translate the health sector's strategic vision into focused, actionable priorities. These objectives are designed to improve access, quality, responsiveness, sustainability, and equity in health service delivery. Together, they form a roadmap that supports Ethiopia's overall mission to reduce morbidity, mortality, and disability and to improve the health status of its people.

4.1. Strategic Objective – C1: Improve Access to Health Services

This objective targets improved accessibility and utilization of health services, especially for vulnerable populations such as mothers, children, adolescents, and youth. It covers key areas like nutrition, hygiene, communicable and non-communicable diseases, hospital emergency services, and maternal and newborn care. The strategy leverages the Health Extension Program for basic services and uses health centers and hospitals as referral and specialized care hubs. Emphasis is placed on PMTCT, TB detection, skilled delivery attendance, malaria control, family planning, antenatal care, and WASH integration. The desired outcome is greater public confidence in the health system, encouraging proactive health-seeking behavior and leading to improved overall health.

4.2. Strategic Objective – C2: Improve Community Ownership

This objective focuses on empowering communities to take an active role in health governance. It promotes awareness, behavioral change, and participation in planning, policy-making, service regulation, monitoring, and resource mobilization. Community Health Promoters, often recruited from model households, will support this by engaging in health promotion and prevention at the household level. The aim is to create sustainable health programs through strong community involvement and leadership.

4.3. Strategic Objective – F1: Maximize Resource Mobilization and Utilization

This objective is about enhancing domestic and international resource mobilization, improving efficiency, and promoting financial equity. It includes mechanisms like risk pooling, facility-based revenue generation, performance-based financing, and implementation of community-based and social health insurance (CBHI and SHI). Strengthening financial management, capacity building, and transparent use of resources are core to this strategy. The goal is to ensure sustainable financing,

equitable distribution, and increased accountability while providing financial protection for citizens.

4.4. Strategic Objective – P1: Improve Quality of Health Services

This objective ensures that health services are delivered according to defined standards across all levels of care. Key aspects include service timeliness, ethical conduct, integration of vertical programs, patient safety, and availability of essential inputs like human resources and supplies. The expected result is a health system that delivers high-quality, safe, and responsive care that meets the community's expectations and needs.

4.5. Strategic Objective – P2: Improve Public Health Emergency Preparedness and Response

This objective emphasizes strengthening early warning systems, preparedness, and response capabilities for health emergencies such as epidemics, malnutrition, and disasters. Key strategies include multisectoral coordination, community engagement, ICT integration, training, and operational readiness. The intended result is a health system capable of early detection, rapid response, and effective recovery from public health threats.

4.6. Strategic Objective – P3: Improve Pharmaceutical Supply and Services

This objective aims to ensure continuous and equitable availability of affordable and quality pharmaceuticals. It includes enhancing drug procurement, logistics, distribution, rational drug use, and minimizing waste. Infrastructure improvements, integration of logistics with health information systems (LMIS-HMIS), and operational research will support better forecasting, quantification, and distribution. The goal is to have the right medicine available at the right place and time in usable condition.

4.7. Strategic Objective – P4: Improve Regulatory System

This objective focuses on enforcing standards and regulations to ensure safety, quality, and public confidence in health services. It includes regulating health practices, products, facilities, waste management, and professional conduct. Through capacity building and implementation of the Health and Health-Related Regulatory Core Process, the health sector aims to create a safe environment with effective oversight of all regulatory aspects.

4.8. Strategic Objective – P5: Improve Evidence-Based Decision Making: Harmonization and Alignment

This objective aims to strengthen data-driven planning, policy-making, and resource allocation through improved monitoring and evaluation (M&E), health management information systems (HMIS), research, and surveillance. Integration of vertical programs and alignment with national systems will enhance coordination. The goal is to adopt “one plan, one budget, one report” principles and use high-quality evidence to guide all health system functions.

4.9. Strategic Objective – CB1: Improve Health Infrastructure

This objective focuses on the construction, maintenance, and equipping of health facilities, as well as the adoption of appropriate technologies. Responsibilities are decentralized to federal, regional,

zonal, and woreda levels. Infrastructure for pharmaceuticals and vaccine production is also included. The expected outcome is a network of accessible, well-maintained, and well-equipped health facilities that serve communities effectively.

4.10. Strategic Objective – CB2: Improve Human Capital and Leadership

This objective covers leadership development, workforce planning, recruitment, performance management, and retention strategies. It promotes intersectoral collaboration, private sector engagement, and the use of ICT in health education. The aim is to build a motivated, skilled workforce distributed equitably across regions and capable of delivering high-quality services.

Chapter 5: Performance Measures and Strategic Initiatives of HSDP IV

This section of HSDP IV outlines how the health sector will track its progress through defined **performance indicators, targets, and strategic initiatives**. These elements are critical for translating strategic objectives into measurable results. The focus is on both impact-level results (e.g., reductions in mortality or disease prevalence) and system-level improvements (e.g., service coverage, financing, and governance).

5.1. Sector Core Performance Indicators & Targets

The health sector has established specific performance targets across priority areas such as maternal and newborn health, child health, HIV/AIDS, TB, malaria, nutrition, and health service infrastructure. These targets aim to significantly improve key health outcomes (e.g., maternal mortality rate to 267/100,000 live births, under-five mortality to 68/1,000, and ART coverage for eligible patients to 95%). Services will be delivered through a tiered infrastructure system—health posts, health centers, primary and general hospitals, and specialized hospitals—backed by systems like the Health Extension Program (HEP), supply chain management, regulatory mechanisms, health financing, and continuous quality improvement.

5.2. Strategic Directions

HSDP IV builds upon past progress and introduces new or refocused directions in key areas including the Health Extension Program (HEP), quality of care, civil service reform, and critical programs. These directions focus on improving service delivery, efficiency, and inclusiveness—especially in underserved and emerging regions. Health infrastructure and ICT expansion are emphasized, as is the integration of climate change preparedness and gender equity in all aspects of planning and implementation.

5.3. Strategic Objectives, Targets, and Initiatives

Each strategic objective is accompanied by measurable five-year targets and initiatives aimed at achieving them. Objectives include improving access to health services, enhancing community ownership, maximizing resource mobilization, improving service quality, strengthening public health emergency responses, ensuring pharmaceutical availability, enforcing regulatory standards, promoting evidence-based decision making, developing health infrastructure, and building human capital and leadership. Initiatives range from scaling up training and telemedicine to improving financial systems, expanding insurance coverage, and enhancing data use for planning and evaluation.

Chapter 6: Costing and Financing of HSDP-IV

The Government of Ethiopia and its development partners have made considerable progress towards achieving the MDGs. However, there still remain major bottlenecks that need to be tackled in the next five years of HSDP IV implementation. Evidences so far strongly point out that the primary obstacles against fast and sustainable targeted health gains are a lack of resources and weak implementation capacity. Therefore, to attain the MDGs in the coming five-year period, the health expenditure needs to be increased and the HSDP IV costing is prepared taking this into consideration.

6.1. Costing Methodology

Costing of HSDP IV is conducted using the Marginal Budgeting for Bottlenecks (MBB) tool, which is an evidence-based result oriented planning and budgeting tool that utilizes knowledge about the impact of existing interventions on health in a country.

MBB is a powerful analytical tool that aims to set interventions and their targets by assessing the health sector impediments to faster progress, identifying ways of removing them, and estimating both the costs of removing them and the likely effects of their removal on MDG outcomes & impacts, such as child and maternal mortality.

MBB includes a generic list of health interventions that have a high impact on child mortality or maternal mortality. A bottleneck analysis has been undertaken, focusing on those that can be delivered via the three service delivery modes: family/community based, population-oriented that are schedulable and individual clinical care.

Family-oriented community-based services	Family Preventive/WASH Services, Family neonatal care, Infant and child feeding, Community Management Illnesses
Population oriented schedulable services	Preventive care for adolescents and adults, Preventive pregnancy care, HIV/AIDS prevention and care, Preventive infant & child care
Individual oriented clinical services	Clinical primary level skilled maternal & neonatal care, Management of Illnesses at Primary Clinical Level, Clinical first referral illness management

6.2. Costing Scenarios & Assumption for HSDP IV

The costing estimate uses two scenarios and each constitutes detailed costing analysis and results. A proportion of bottleneck reduction is considered to formulate the two Scenarios with a separate set of coverage targets.

The base-case scenario:

- Assumes that there would be no compromising of the plan to achieve the MDGs by the year 2015. This scenario considers universal access to health centre, staffing, equipping and availing drugs and supplies to HCs as per the standard. There would be strengthening for supply chain management system to make HCs fully functional and provision of Basic Emergency Obstetric Care and essential newborn care. In addition, selected HCs would be upgraded to primary hospitals to provide ambulatory and inpatient services, including emergency surgery, caesarean section and blood transfusion services. Existing general and specialised hospitals would also be strengthened.
- For services, the base-case scenario aims at reaching: BEmONC and CEmONC services in HCs & hospitals - 100%; ART treatment - 85%; Full immunisation - 90%; Coverage in clean and safe delivery by HEWs - 38%; Coverage of deliveries by skilled attendants - 62%; Model Household graduation - 85%, Latrine utilisation - 82%, ITN ownership and use by households - 90% & 86%, respectively.

The best-case scenario:

- This scenario follows the MDGs Needs Assessment (2005) that assumes a condition of no financial constraint. It projects fulfilling the commitment of the health sector to improve and sustain quality of health service delivery, embracing prevention and control of communicable diseases, and improving the quality of clinical care for acute and chronic health problems. It also assumes ensuring universal access to quality clinical care, with access to advanced treatment at all levels of the health care delivery system. The best-case scenario focuses mainly on evidence-based high level clinical care and the protection of patient rights in accessing the needed health care services.

	Base-case Scenario	Best-case Scenario
Under five mortality	33.8%	46%
Maternal mortality	54.8%	56.6%
Cost per capita per year in US\$	11.96%	13.96
Total budget for the 5 years US\$	8.83 B	10.828 B
Average yearly increase to achieve it	9%	13.5%
Strengthening service delivery	48%	56%
Expansion and strengthening health infrastructure	37%	31%
Leadership and governance	15%	13%
The 3 major programs (prevention and control of malaria, HIV/AIDS, and TB and leprosy) from the total	30%	40%

To implement HSDP IV, an estimated US\$8.83 billion (base-case) to US\$10.828 billion (high-case) is required, but significant financing gaps exist. Putting the economic level of the country, the gap needs to be filled through resource mobilization, the main part being from development partners. Development partner funding shows a decline in later years and this results in substantial and widening funding gaps of US\$4.337 billion (base-case) and US\$6.339 billion (high-case) over five years.

6.3. HSDP IV Financing Channels

HSDP IV utilizes multiple financing channels to ensure funds reach the health sector effectively.

- Channel 1a (Un-earmarked): Government-controlled funds disbursed through specialized finance bodies at each administrative level, primarily used for budget support.
- Channel 1b (Earmarked): Funds from donors tagged and sent directly to regions/zones/woredas for specific, donor-approved activities.
- Channel 2: Funds managed directly by sector units, bypassing regional/zonal finance bodies with disbursements variations.
- Channel 3: Donors handle procurement and payment directly to contractors, with the government ensuring the expenditures align with HSDP goals.

For the health sector, the preferred modes are mainly i) the block-grant to woreda and ii) the MDGs Performance Fund. These channels are preferred due to less transaction cost on the government.

6.4. Application of “one-budget” to the different channels and mechanisms

The main purposes of implementing “one budget” at all levels of the health system are

- 1) to make sure that various channels and funding mechanisms finance “one plan” and
- 2) to reduce transaction cost of the government because of having to deal with the various channels and financing mechanisms.

Implementation Strategies include:

- Aligning project support with sector priorities.
- Sharing project information with local governments for integration into local plans.
- Facilitating negotiations between local governments and project managers for flexibility and avoiding duplication.
- Conducting joint monitoring and evaluation of projects with local government and stakeholders.

Chapter 7: Management Arrangement of HSDP IV

This describes the mandates of the different levels of the health system. It also gives elaborations to the planning, governance, procurement, financial management, monitoring and evaluation processes that applies to HDP IV.

7.1. Governance Structure

Governance of the HSDP IV is characterized by a tiered coordination system involving Federal, Regional, Zonal, and Woreda levels. Specific governance platforms include:

- Country Joint Steering Committee (CJSC)
- Regional Joint Steering Committee (RJSC)
- Woreda Joint Steering Committee (WJSC)

These ensure alignment of strategic and operational health priorities and harmonize stakeholder inputs in decision-making processes.

This also requires inter-sectorial collaboration with MOE, Ministry of Water Resources, Ministry of Agriculture on nutrition, Ministry of Finance, media, Ministry of Youth and Sports, Ministry of Women Affairs, Ministry of Transport and Public Private Partnership.

7.2. Health Service Delivery Arrangement

The Ethiopian health service is restructured into three tier system.

A) Primary Health Care Unit (PHCU)

PHCU is composed of a HC and five satellite HPs. These provide services to 25,000 population altogether. HP is staffed with two HEWs, whereas a HC is staffed with an average of 20 staff. It provides both preventive and curative services. It serves as a referral center and practical training institution for HEWs.

B) Primary Hospitals

Staffed by an average of 53 professionals it provides inpatient and ambulatory services to an average population of 100,000 providing an emergency surgery service. It also serves as a referral center for HCs and a practical training center for nurses and other paramedics.

C) General Hospital

Provides inpatient and ambulatory services to an average of 1,000,000 people. It is staffed by an average of 234 professionals, serving as a referral center for primary hospitals.

D) Specialized Hospital

Serves an average of five million people. It is staffed by an average of 440 professionals. It serves as a referral general hospitals.

7.3. Planning

The Ethiopian health planning is composed of two planning cycles. The first and the single most reference planning cycle is **The Five Year Strategic Planning** called **HSDP**, serving as a blue print on which all Regional Health Planning and developed. The second is **The Annual Planning** that translates the five year strategic planning into details of achievable targets, strategies and interventions under the different levels of the health care system.

The planning process is both **top-down and bottom-up**, with that of the Regions and Woreda's. ensuring alignment with national strategies while integrating local priorities. The planning phases of annual plans include:

- **Core Plan:** Focuses on achieving national health targets.
- **Comprehensive Plan:** Combines core and locally specific activities. It has features such as: *Scope, Resource mapping and Financing, Implementation schedule and Monitoring framework.*

7.4. Monitoring and Evaluation (M&E)

This is an action-oriented and preplanned management tool that operates on adequate, relevant, reliable and timely collected, compiled and analyzed information on programme/project objectives, targets and activities. The objectives of M&E are to improve the management, optimum use of resources of programme and to make timely decisions. It supports evidence-based decision-making through the following:

- a) Routine Administrative Reporting (HMIS):** Upgraded system to improve data quality.
- b) Performance Monitoring and Quality Improvement:** Internal audits, reviews, and tracking against performance indicators.
- c) Evaluation and Operational Research:** Assesses program effectiveness and informs adjustments.
- d) Integrated Supportive Supervision (ISS):** Enhances health worker performance through regular feedback.
- e) Inspection:** Ensures compliance with standards.

7.5. List of Key Indicators for Result Framework at National Level

Indicators measure progress towards the agreed objectives and targets. The performance framework consists of **quantitative indicators** that reflect progress in:

- **Health outcomes:** Maternal, child mortality, epidemic control.
- **Service delivery:** Coverage of key interventions.
- **Resource utilization:** Budget allocation and expenditure ratios.
- **Data quality:** Completeness and timeliness of reports.
- **Health facility performance:** Utilization of new HMIS tools and reporting compliance.

These indicators serve as benchmarks at national, regional, and facility levels

7.6. Risk Mitigation

One of the major risks the HSDP IV can face is inadequacy in financial resources. Major Risks include:

- **Funding gap:** An estimated **US\$ 4.34 billion** shortfall under the high-case MDG scenario.
- **Weak private and community engagement.**
- **Supply-side inefficiencies** (procurement, logistics)
- **Demand-side limitations** (low health-seeking behavior, cultural barriers).

Mitigation Strategies:

- **External resource mobilization:** Via donor mechanisms and the MDG Performance Fund.
- **Health Insurance schemes:** To improve domestic financing.
- **Strengthened PPP and community ownership:** Through inclusive health governance.
- **Enhanced planning and supervision:** To prevent service gaps and inefficiencies.
- **Stakeholder collaboration:** Leveraging multi-sectoral engagement for risk sharing and innovation.

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